



CLINICAL PERFORMANCE LAB
The Cheat Sheet Library

VOL IV OF IV

Management Plan & SOAP Note

The faculty-scored 20% — and how
to avoid the harmful-to-patient flag

The Plan section is where faculty actually read your work. Get this
part right and you can compensate for weaker history/PE scores.

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Built by clinicians. Verified against live platform data.

Vol IV of IV

Management Plan & SOAP Note

The faculty-scored 20% — and how to avoid the harmful-to-patient flag

Here's the thing about the Management Plan: it's the only section that's 100% faculty-scored.

Auto-grading stops at the differential. From there, a human nursing faculty member reads what you wrote and assigns the remaining 20% of your grade based on the quality of your plan, your medication choices, your patient education, and your follow-up scheduling.

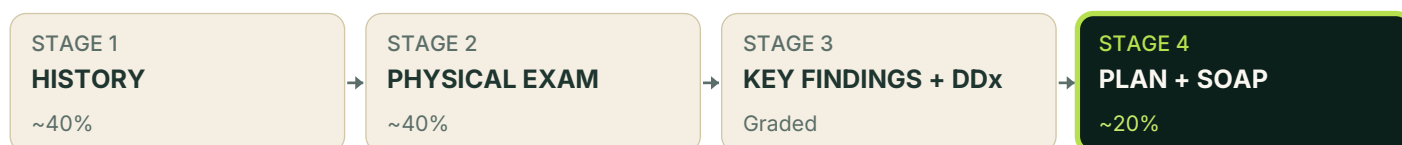
Faculty are looking for evidence-based practice (EBP), correct medication dosing, appropriate referrals, and a follow-up timeline that matches the diagnosis severity.

They're also looking for one specific trap: ordering harmful tests. iHuman flags certain tests as "harmful to patient" on certain cases — and one trigger can drop your Tests score to zero.

This sheet covers the 6-part management plan structure faculty look for, the harmful-flag trap, the medication dosing reference, and the SOAP note template that scores well across programs.

Apply it across every case.

THE IHUMAN CASE FLOW · You are here



The Harmful-to-Patient Flag

⚠ Critical Scoring Trap

This is the single biggest scoring trap in iHuman.

If you order a test that the platform considers harmful for that patient — too aggressive, too invasive, contraindicated, or unnecessary given the clinical picture — the platform flags it and your Tests score drops to 0%.

Classic examples:

Migraine with aura, normal exam, no red flags → Don't order CT or MRI. Clinical diagnosis. Ordering imaging = harmful flag.

Headache with negative SNOOP4 → Same. Imaging is not indicated.

Stable URI → Don't order chest X-ray.

Simple viral gastroenteritis in a child with normal vitals → Don't order CBC, BMP. Clinical management.

The rule: Order tests when they will change your management. If a test result wouldn't change what you do next, don't order it. The phrase to remember from Choosing Wisely: "More tests, more problems."

The 6-Part Management Plan

THE 6-PART MANAGEMENT PLAN · Faculty look for all six

Dx DIAGNOSTICS

Tests ordered — or 'no tests needed' with rationale

Rx PHARMACOLOGIC

Name, dose, route, frequency, duration, dispense qty, refills

Non-Rx NON-PHARM

Lifestyle, diet, exercise, sleep, behavior change

Ref REFERRALS

Specialty, urgency, reason

Edu EDUCATION

Disease process, warning signs, when to return

F/U FOLLOW-UP

When + monitoring parameters + ER return precautions

Why Each Part Matters

Faculty expect all six domains covered. Each scores separately.

Missing any section flags incompleteness. Even a brief sentence per section is better than skipping one.

Medication Documentation — Six Required Elements

Every medication needs all six elements or it gets flagged as incomplete:

- 1. Name** (generic + brand if relevant)
- 2. Dose** (e.g., 10 mg)
- 3. Route** (PO, IM, IV, topical)
- 4. Frequency** (e.g., daily, BID, TID, QID, PRN)
- 5. Duration** (e.g., 10 days, 30 days, ongoing)
- 6. Dispense quantity** and **refills** (e.g., #30, 2 refills)

Skipping the dispense/refill quantity is a frequent point-loss item — faculty look for it.

Medication Answer Key (by case)

iHUMAN MEDICATION ANSWERS · by case template

▲ FOR iHUMAN CASE DOCUMENTATION ONLY

Verify all dosing with current Epocrates/UpToDate before any clinical application.

STAGE 2 HTN (HARVEY/FELIPE/HERBIE/ANSELMO)

Lisinopril 10 mg + Hydrochlorothiazide 25 mg

PO daily, dispense #30, 2 refills (each)

Two-drug therapy required per ACC/AHA 2017

GAS PHARYNGITIS (AMANDA WHEATON, ET AL.)

Penicillin V 500 mg

PO BID × 10 days, dispense #20, 0 refills

10-day course mandatory. Amoxicillin 500mg BID acceptable alt.

MIGRAINE ACUTE (BEBE/MARTA/JESSICA/MABEL)

Sumatriptan 50 mg

PO at onset, may repeat × 1 after 2h; dispense #9, 0 refills

Max 200 mg/day. ≤10 days/month. Hold if pregnant.

MIGRAINE ADJUNCT NAUSEA

Metoclopramide 10 mg

PO PRN nausea, max 45 mg/day; dispense #20, 0 refills

Often paired with Sumatriptan.

MIGRAINE NSAID OPTION

Naproxen sodium 500 mg

PO once, may repeat once; dispense #18, 0 refills

Max 1000 mg/day. Alternative to triptan.

HYPERLIPIDEMIA (CYNTHIA FRANCIS)

Atorvastatin 20 mg

PO at bedtime, dispense #30, 2 refills

Continue thyroid hormone replacement concurrently.

ADHD INATTENTIVE (KENNEDY POOLE)

Methylphenidate ER 18 mg

PO morning, dispense #30, 0 refills, Schedule II

Baseline ECG before start. Titrate q2 weeks.

PEDIATRIC VIRAL GE (SAMANTHA/COURTNEY)

Oral rehydration solution (Pedialyte)

Small frequent sips, replace fluid losses

No antiemetics in mild dehydration. Return for any worsening.

Doses reflect what the iHuman platform expects on listed case templates. Not for clinical use.

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EBP References — What Faculty Cite

Most programs expect 2–3 scholarly references in APA format. Faculty want recent, authoritative sources.

Tier 1 — Always acceptable:

Clinical practice guidelines (e.g., ACC/AHA, GINA, ADA, IDSA)

UpToDate (cite the topic and date accessed)

Peer-reviewed journal articles (within last 5 years)

AAP guidelines for pediatric cases

Tier 2 — Acceptable but secondary:

Textbooks (current edition) — Bates, Dunphy, Hollier

CDC guidelines

WHO recommendations

Avoid:

Wikipedia

Medscape (use sparingly — not all programs accept)

Patient-facing health blogs

Pharmaceutical company websites

Format reminder: APA 7th edition. In-text citation: (Author, Year). Reference list at the end.

Follow-Up Timeline by Diagnosis Severity

- ✓ **Acute, self-limiting (URI, viral GE)** — 7–14 days or PRN if not resolving
- ✓ **Acute, treatable (GAS pharyngitis, UTI)** — 7–10 days to confirm resolution
- ✓ **Chronic, controlled (stable HTN on meds, DM)** — 3 months
- ✓ **Chronic, new or uncontrolled (newly diagnosed HTN, new DM)** — 2–4 weeks
- ✓ **Severe / pre-treatment (Stage 2 HTN before meds adjusted)** — 1–2 weeks
- ✓ **Mental health (new SSRI start)** — 2 weeks for tolerability, 4–6 weeks for efficacy
- ✓ **Pediatric ADHD med initiation** — 2–4 weeks to titrate, then monthly until stable
- ✓

- ✓ **Always document ER warning signs** — chest pain, severe headache, vision changes, dyspnea, BP > 180/120, fever > 102 with neck stiffness, etc. (case-specific).

The SOAP Note Template

S — Subjective:

Chief complaint (in patient's own words, in quotes)

HPI in narrative form, organized by OLDCARTS

PMH, FH, SH (brief, relevant)

ROS (positives and significant negatives only)

O — Objective:

Vitals

General appearance

PE findings by system (positives and pertinent negatives)

Test results if available

A — Assessment:

Primary diagnosis (with ICD-10 if required by program)

Differential diagnoses (3–5, ranked)

Brief diagnostic reasoning (1–2 sentences)

P — Plan:

Diagnostics, pharmacologic, non-pharm, referrals, education, follow-up (the 6-part structure above)

Reference list in APA format

Common Plan Traps That Cost Faculty Points

⚠ Ordering harmful tests

The biggest single trap — drops Tests score to 0% if triggered. Always ask: will this result change my management?

⚠ Incomplete medication orders

Missing dose, route, frequency, duration, dispense quantity, or refills. Faculty look for all six.

⚠ Forgetting NSAID counseling on hypertension cases

NSAIDs raise BP. If your HTN patient is taking ibuprofen, you must D/C and switch to acetaminophen. Skipping this loses points.

⚠ Skipping patient education on warning signs

Faculty want to see explicit ER return precautions for every diagnosis.

⚠ Wrong follow-up timeline

Stage 2 HTN with new meds → 2 weeks, not 3 months. Match severity to interval.

⚠ Generic references

"WebMD" or "Mayo Clinic patient portal" gets flagged. Use clinical guidelines and peer-reviewed sources.

⚠ Missing OSA referral on HTN cases

Snoring + HTN + male/overweight = sleep study referral. Scored as a separate management item.

Next steps

You've completed the CPL Cheat Sheet Library. Volumes I-IV cover the universal patterns that apply across every iHuman case.

READY FOR THE FULL GUIDE?

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